

WeShare Legacy[®]

Summary of Sharing

Program Description

WeShare Legacy provides peace of mind to 65+ Members looking for added support to help with medical expenses left after Original Medicare. There's no more stressing over a potentially limited provider network as WeShare Legacy provides sharing benefits when a Member cannot locate a Medicare specialist provider with our expansive PPO network. With low monthly contributions and Annual Member Care Share, Legacy offers added security for Member's on a fixed budget. WeShare Legacy works alongside your Medicare plan to help provide faith-based support to proactively manage your whole health: mind, body, and spirit.

Eligible sharing services include:

- Bills where Medicare applies Member liability (co-pay, co-insurance, or deductible);
- Medical services not standardly covered by Medicare (e.g. Acupuncture or Chiropractic care for any condition), as long as UHSM allows the service;
- Services beyond Medicare's limits where WeShare Legacy offers broader sharing limits
- Services for professional providers who are not registered with Medicare (when the type of specialist provider needed is not available in the Medicare network within 30 miles of the Member's home)

Note: WeShare Legacy does not share in services that Medicare deems ineligible due to medical necessity, lack of pre-certification when required by Medicare, or a provider contracting issue. WeShare Legacy also does not share in services where Medicare does not hold the Member liable.

Material Links

Program Overview	Program Brochure	Program Guides: N/A	Membership Guidelines
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Available Markets

States where we sell the program

All States except WA, MT, ND, MA, AK, HI

Network

See specific line item details for sharing levels

Professional Services	Facility / Hospital Services	Walk-in Clinic	Virtual Care
Medicare Registered Providers that accept Medicare assignment: Medicare network and pricing (medical bill must be submitted to Medicare first, and will crossover to UHSM. If crossover fails, provider can submit to UHSM/through Change Healthcare with the Medicare EOB {MEOB} for consideration) Medicare Registered Providers that do not accept Medicare assignment: 115% of Medicare allowance (medical bill must be submitted to Medicare first, and will crossover to UHSM. If crossover fails, provider can submit to UHSM/through Change Healthcare with the Medicare EOB {MEOB} for consideration) Non-registered Providers: PHCS network (only allowed in certain circumstances when a Medicare provider is not available within 30 miles of the Member's home). Bills should be submitted through UHSM's standard billing process, preferably electronically through our Payor ID with Change Healthcare (UHSM1). Note: Associates should leverage the Medicare provider finder to confirm unavailability of a Medicare provider and review the sharing benefits and pre-cert requirements for the service before approving or suggesting the use of a PHCS provider, to ensure eligible for a PHCS provider. Medicare providers Medicare Medical Equipment Suppliers	Medicare Network and Pricing: Facility bills must be submitted to Medicare for consideration as primary. WeShare Legacy will consider facility services after Medicare has processed and considered their coverage, leveraging the Medicare allowed amount. Bill must be submitted to Medicare first, and will crossover to UHSM/through Change Healthcare. If crossover fails, provider must submit to UHSM/through Change Healthcare with the Medicare EOB {MEOB} for consideration.	While we do not have a specific agreement with a walk-in clinic for this program, CVS MinuteClinic does participate with Medicare and Members may leverage their services. Bill must be submitted to Medicare first, and will crossover to UHSM/through Change Healthcare. If crossover fails, provider can submit to UHSM/through Change Healthcare with the Medicare EOB {MEOB} for consideration.	Medical and Behavioral Health: Amwell Behavioral Health: BetterHelp

Out-of-Network / Non-Network Providers

WeShare Legacy Members are encouraged to seek care from participating, in-network care providers through Medicare or WeShare's expansive network. However, WeShare may, at its discretion and for eligible medical services, consider a non-participating provider eligible if the following conditions are met:

- Services are not covered by Medicare leaving a Member liability or a Medicare provider was not available to perform the services within 30 miles of the Member's home (pre-approval by Member Services required);
- A participating provider is not available through the PHCS network within 30 miles for the needed treatment or service (pre-approval by Member Services required);
- Services rendered were considered emergent, or;
- Services are not emergent but the Member received services at an in-network facility where certain out-of-network providers rendered care (e.g. anesthesiologist). Note: for Medicare claims/registered providers, we will follow Medicare allowances/will not allow more than Medicare allows/holds the Member liable for.

Pre-certification Requirements Summary

See specific line item details for pre-certification details for each service

Pre-certification is not required for any services processed by Medicare as primary (Members/providers should obtain pre-certification through Medicare as needed).

Pre-certification is required for the following professional services if the Member plans on leveraging the PHCS network (when a Medicare provider is not available):

- Surgeries not performed in the doctor's office
- Certain surgeries, regardless of the place of service: Arthroscopy or joint surgery, Bladder repair, Cardiac Cath/Angioplasty, Deviated septum/nasal surgery, Laminectomy/spinal surgery, Implants, Transplants (except corneal), Mammoplasty, Blepharoplasty, Varicose veins stripping and ligation
- Endoscopic, laparoscopic, and colonoscopy procedures
- Imaging such as EBCT (Electron Beam Tomography), CT scan, MRI, and PET scans
- Chemotherapy and radiation
- Trigger point or pain management injections, infusion therapy, biologic drugs, ablations, stimulators, blocks and any drugs >\$1,500 a dose performed by PHCS providers as a one-time service (on-going treatment must be rendered by a Medicare provider)
- Physical, Occupational, and Speech Therapy
- Tobacco related illnesses

Pre-cert is also required for facility services if Medicare benefits have been exhausted for the service (e.g. inpatient days or skilled nursing days exhausted).

WeShare Legacy reserves the right to exclude services obtained without pre-certification, where required.

Member or Provider Services must pre-screen the services to confirm that pre-certification is required before sending to MedWatch. The case number from the Member/provider's inquiry must be included on the pre-certification submission in order for MedWatch to consider the pre-certification case.

Multiple Services Billed

When multiple services are billed, each service will apply the sharing benefits applicable to that service. For example, if an office visit is billed along with labs, both the office visit and lab sharing benefit will be applied to the Shareable Medical Bill (SMB).

Pre-existing Conditions

None.

Sharing Maximums

Applicable sharing maximums are outlined throughout the document within the line item they apply to. Sharing maximums apply to each individual Member unless otherwise stated. If a sharing dollar maximum applies, the maximum is accumulated with paid sharing dollars only (AMCS and consultation fees do not contribute towards the maximum). Once a sharing maximum has been reached, we will continue to process Shareable Medical Bills (SMB) for the services in order to apply network contracted rates, AMCS, and consultation fees where applicable. However, additional shared dollars will not be paid for services beyond the maximum. Therefore, once a maximum is reached, the Member can still benefit from the discounted rates of our network for the service, as well as continue to accumulate AMCS for the service throughout the year.

Waiting Periods

Includes services where a waiting period applies in order to be eligible for sharing. Does not include services that have a sharing maximum for a certain time period; see Sharing Maximums for such services

None.

Eligibility Requirements

Members must be of Christian Faith, a permanent U.S. resident, 65 years old or older, and enrolled in Original Medicare for Part A & Part B. WeShare Legacy cannot sell to Medicare Advantage members. WeShare Legacy applicants that are transitioning from another UHSM program do not need to be screened by Milliman. Applicants who are new to UHSM are screened and have to be reviewed through the standard Eligibility Review process. Members must agree to our Statement of Shared beliefs as outlined in the Member Guide and commit to living a healthy lifestyle. If, at any time, it is discovered that a WeShare Sharing Member did not submit a complete and accurate medical history within the Membership Enrollment Application, a membership cancellation may be issued retroactively to her / his Effective Membership Date.

WeShare Legacy membership only allows an individual program; other family members must be enrolled separately and under the applicable program for their needs. There is no correlation in programs across family memberships. Children under 26 who may normally be included under the 65+ Member's program may be enrolled separately in a WeShare program and act as the primary member as long as they meet the requirements of WeShare.

Continuity/Continuation of Care

In instances where a Member is receiving care from a provider whose contract with PHCS terminates, continued services are eligible up to 90-days after provider contract termination notice. The Member must notify WeShare that they would like to continue services/ have a continuation of care need. During continuation, associated Shareable Medical Bills (SMB) will be processed as if the network termination had not occurred. However, the Provider may pursue the Member for any amounts above the WeShare program's sharing amount.

For purposes of this provision, "continuing care patient" means an individual who is undergoing a course of treatment for a serious and complex condition from a specific Provider, undergoing a course of institutional or inpatient care from a specific Provider, scheduled to undergo non-elective surgery from a specific Provider, including receipt of postoperative care with respect to the surgery, or was determined to be terminally ill and is receiving treatment for such illness from a specific Provider.

Specialty & Wellness Perks Eligibility:

Dental Discounts - Careington (provider finder)	Yes, except residents in MT, WA, UT or VT where the program is not available
Vision Discounts - Superior Vision (provider finder)	Yes, except residents in MT, WA, UT or VT where the program is not available
LASIK Discounts - Quasight LASIK (provider finder)	Yes, except residents in MT, WA, UT or VT where the program is not available
Hearing Discounts - Amplifon Hearing Health Care	Yes
Nutrition Support - Noom	Yes
America's #1 Meal Kit - HelloFresh	Yes
Exercise Resource - Fitbod , Active&Fit Direct	Yes
Biblical Resource - RightNow Media	Yes

Healthy Discount Program

WeShare Legacy members are NOT eligible for the Healthy Discount Program.

Renewal Discount Program

WeShare Legacy members are NOT eligible for the Renewal Discount Program.

Add-on Programs

Programs that can be added to this program, for an additional fee

N/A, not eligible for S.M.A.R.T. Care or A.I.D.D. Care

Member Cost-Sharing

Membership Fees	Application fee: \$149 Tobacco, marijuana & e-cigarette users: \$50 per usage member, per month Default payments: \$25	
Consultation Fees	None	
Annual Member Care Share (AMCS) The AMCS is selected during enrollment and can be found in Salesforce (for service and SMB: on the 360 view of the SMB tab titled Individual Max/AMCS; for sales: on the home screen next to the program name) Legacy policies are sold as individual only, there is no family AMCS. To meet the family amount, a minimum of two Members must meet their individual AMCS	Individual	\$500

Medicare Responsibility

Note: This is for internal information only; Members should be directed to Medicare to understand their benefits

Medicare Part A Deductible	2025: \$1676 2026: \$1,736
Medicare Part B Deductible	2025: \$257 2026: \$283
Medicare Copayments	Inpatient and Skilled Nursing Facility Copayments apply per day for certain days
Medicare Coinsurance	Part B 20%

Pharmacy

Pharmacy Network	While Pharmacy sharing benefits are not available on WeShare Legacy, Members have access to Optum Perks - a discount program Member's can elect to use instead of any other Pharmacy benefits they may have (e.g. Medicare Part D). It cannot be used in conjunction with any Pharmacy insurance. Members should ask their Pharmacist if the Optum Perks discount program will yield lower cost medications than their insurance to decide. If a Member does not have Pharmacy insurance, they are able to leverage the discount program to save on their drug costs. See perks.optum.com/UHSM for more information.		
Annual Member Care Share (AMCS)	N/A		
Annual Pharmacy Share Maximum	N/A		
	Retail Pharmacy, up to 30-day supply Fee applies per prescription	Mail-Order, up to 90-day supply Fee applies per prescription	Limitations and Exclusions
Formulary Generic Medication	N/A	N/A	N/A
Standard Brand Formulary Medication	N/A	N/A	N/A
Non-Formulary Brand Medication	N/A	N/A	N/A
Specialty Brand Medication	N/A	N/A	N/A

Preventive Care

Services must be billed with a routine, preventive procedure and/or diagnosis code, rendered by a Medicare registered provider, and submitted to Medicare first to be considered preventive and processed according to this section. UHSM will process once Medicare processes and share routine services in full that Medicare denies and holds the patient liable for, or processes and leaves some Member responsibility. If Medicare does not hold the Member liable, UHSM will not share in the service. If diagnostic services are billed in conjunction with preventive care, the preventive care will process according to this section but the diagnostic care will process according to the service billed as non-routine. Except where services are excluded, UHSM follows the healthcare.gov guidelines to help categorize preventive care.

Note: if a Member disagrees with the processing (e.g. advises it was a routine service but is not billed as such), Member Services should explain what is included as routine. If the Member confirms diagnostic services were not performed, contact the provider to discuss if a corrected claim needs to be filed with Medicare. Once the corrected claim is processed by Medicare, UHSM will reprocess as needed once a corrected claim with a corrected Medicare EOB is submitted.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Routine Annual Physical	100%	\$0	Does not cover a routine annual physical. Within the 1st 12-months of Medicare membership, Part B covers a "Welcome to Medicare" preventive visit in full. This visit includes: Review of medical and social history; discussion on preventive services, including certain screenings, shots or vaccines (like flu, pneumococcal, and other recommended immunizations); height, weight, BMI and blood pressure measurements; referrals for other care as needed; a simple vision test; depression risk assessment; discussion on creating advance directives. After 12-months of eligibility, Part B covers a yearly "Wellness Visit" in full, which includes: Review of medical and family history; review of a screening schedule for recommended preventive services; height, weight, and blood pressure measurements; review of current prescriptions; personalized health advice; advance care planning; and an optional "Social Determinants of Health Risk Assessment" to help understand social needs and their impact on treatment. Part B covers an annual depression screening in full.	No	None	None	To contain costs and save multiple trips to the doctor, Members are encouraged to have their routine physical performed as part of their "Welcome to Medicare" preventive visit or their yearly "Wellness Visit". While Medicare does not cover a full annual physical, physicals should still be billed to Medicare first to determine what portion of the service they will allow. UHSM will consider the services once Medicare processes and allow denied services that Medicare holds the Member liable for.
Routine Annual Well-Woman Visit	100%	\$0	Part B covers Pap test, Pelvic, and Breast exam in full every 24-months unless high risk for cervical or vaginal cancer, when it is covered once every 12-months.	No	None	None	Allowed every 24-months, unless a Member is considered high risk for cervical or vaginal cancer when it is allowed annually.
Routine Bone Density Screenings	100%	\$0	Part B covers bone mass measurement tests/ scans in full every 24-months in certain conditions: • Member is female and doctor has determined risk for osteoporosis based on estrogen levels, medical history, and other findings • X-rays shows possible osteoporosis, osteopenia, vertebral fractures • Member takes or is prescribed prednisone or steroid-type drugs • Member has been diagnosed with primary hyperthyroidism • Being monitored to ensure osteoporosis drug therapy is working	None	None	None	Allowed every 24-months

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Routine Cervical Cancer Screenings	100%	\$0	Part B covers Pap test, Pelvic, and Breast exam in full every 24-months unless high risk for cervical or vaginal cancer, when it is covered once every 12-months.	No	None	None	Allowed every 24-months, unless a Member is considered high risk for cervical or vaginal cancer when it is allowed annually. Note: Colposcopies (biopsy of the cervix) or other cervical incisions are considered diagnostic, refer to surgery line item.
Routine Colon Cancer Screenings Includes anesthesia or medications given to lessen discomfort if also billed with routine diagnosis codes.	100%	\$0	Part B covers fecal occult blood tests in full annually. Part B covers blood-based biomarker screening test or multi-target stool DNA tests (e.g. Cologuard) in full to check for colorectal cancer every 3-years under the following conditions: Member is 45–85 years-old with no symptoms of colorectal disease (including, but not limited to, lower gastrointestinal pain, blood in stool, or positive guaiac fecal occult blood test or fecal immunochemical test) and are at average risk for developing colorectal cancer, meaning: • No personal history of adenomatous polyps, colorectal cancer, or inflammatory bowel disease (including Crohn's Disease and ulcerative colitis). • No family history of colorectal cancers or adenomatous polyps, familial adenomatous polyposis, or hereditary nonpolyposis colorectal cancer. Part B covers barium enemas every 4-years (for average or high risk) if the doctor recommends it in place of a colonoscopy or sigmoidoscopy. It is allowed every 2-years if the Member is high risk. However, the Member pays 20% coinsurance or their hospital copay, depending on the place of service. Part B covers routine colonoscopy in full every 24-months if high risk for colorectal cancer, every 10-years if not high risk, or every 4-years after previous flexible sigmoidoscopy with no minimum age requirement. 15% coinsurance applies for polyp removal (when found as part of a routine colonoscopy). Part B covers flexible sigmoidoscopy in full every 4-years or 10-years after a previous colonoscopy screening (when average/low risk). 15% coinsurance applies for lesion or growth removal (when found as part of a routine sigmoidoscopy).	No	None	None	To help contain costs, Members are encouraged to request a colon cancer screening as part of their annual physical. The following frequency stipulations apply: • Self-Screen Option: Cologuard/biomarker screening tests are eligible every 3 years • Flexible Sigmoidoscopy: Eligible every 4 years • Colonoscopy: Eligible every 10 years Polyp, abnormal tissue removal and biopsies are considered diagnostic and subject to AMCS, whether performed in conjunction with a screening or as a separate procedure. See the surgery line item for complete sharing benefits.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Routine Immunizations	100%	\$0	Part B covers the following in full: Flu shots annually, Covid-19, Hepatitis B (for medium to high risk Members), Pneumococcal / pneumonia Part D covers vaccines that the Advisory Committee on Immunization Practices (ACIP) recommends in full, including but not limited to: Respiratory Syncytial Virus (RSV), Shingles, Tdap, MMR, BCG, Meningococcal, and Hepatitis A and B for low-risk individuals	No	None	None	- To help contain costs, preventive immunizations should be performed in conjunction with an annual physical. Where not possible (e.g. the PCP does not stock the immunization), Members should be encouraged to obtain immunizations from CVS MinuteClinic. The CDC provides a schedule of recommended, routine immunizations: https://www.cdc.gov/vaccines/imz-schedules/index.html - If a vaccine is only covered through Part D and the member does not have Part D, UHSM will process the immunization without the Medicare EOB. - Immunizations obtained strictly for travel purposes are not eligible. See the Program Exclusions section for a list of ineligible travel immunizations. Immunizations obtained as a work requirement are not eligible as work-related services are not eligible. - Non-routine immunizations are normally billed with a non-routine diagnosis and are subject to AMCS (e.g. rabies vaccine after animal bite, tetanus vaccine after an accident, Anthrax vaccine for a high-risk individual).
Routine Labs & Tests	100%	\$0	Part B covers the following in full: - Cardiovascular disease screenings every 5-years (cholesterol, lipid, triglyceride) - Diabetes screening (blood glucose test), up to twice a year for individuals with certain risk factors - Hepatitis B and C for high risk/certain individuals members either annually or once in their lifetime - Sexually Transmitted Disease (STD) testing (HIV, chlamydia, gonorrhea, syphilis and hepatitis B) annually for high risk individuals	No	None	None	- To help contain costs, Members are encouraged to request routine labs as part of their annual physical/ annual wellness visit. - Electrocardiogram (ECG or EKG) are generally not considered routine and are subject to the AMCS. - Testing for STDs is not eligible.
Routine Imaging (Mammograms & Lung Cancer Screenings)	100%	\$0	Part B covers a screening Mammogram in full annually. Part B covers lung cancer screenings (low dose computed tomography) in full annually for high risk individuals (primarily those who have smoked).	No	None	None	To help contain costs, Members are encouraged to request routine imaging services as part of their well-woman visit or annual physical/annual wellness visit.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Routine Prostate Screenings	100%	\$0	Part B covers a PSA test annually in full. Part B covers a digital rectal exam annually, subject to Part B deductible and 20% coinsurance or their hospital copayment, depending on the place of service.	No	None	None	To help contain costs, Members are encouraged to request a prostate screening as part of their annual physical/annual wellness visit.

Diagnostics and Medical Care

Except when a Medicare-registered provider is not available within 30 miles of the Member's home for certain professional services, services must be rendered by a Medicare registered provider and claims must be submitted to Medicare first. UHSM will process eligible services once Medicare processes and share according to the Member's Sharing Program when Medicare denies and holds the patient liable (except for medically necessary/experimental/investigational denials), or when Medicare processes and leaves Member responsibility. If Medicare does not hold the Member liable, UHSM will not share in the service. For Medicare registered providers, UHSM will utilize Medicare's guidelines to determine medical necessity, therefore, services that Medicare deems investigational, experimental, or not medically necessary will not be eligible for sharing. For certain professional services, if there is not a registered Medicare provider within 30 miles of the member's home to perform the needed service, UHSM will consider bills from PHCS PPO providers and process submitted bills without the Medicare EOB (MEOB) for eligible services and based on medical necessity determined by UHSM's Utilization Management partner (when applicable). All hospital/facility bills and services listed as "No" in the "Allowed for Non-Registered Medicare Providers?" column must be submitted to Medicare first; UHSM will only consider once Medicare processes

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Acupuncture	100% after AMCS	AMCS + amounts over annual visit limit	Part B covers for lower back pain only, up to 12 visits in a 90 day period. An additional 8 visits are allowed if the treatment is yielding condition progress in a 12-month period. Services must be performed by a licensed medical provider, not a licensed acupuncturist.	Yes	None	None	Up to 12 annual visits for any diagnosis Acupuncturists from the PHCS PPO network allowed if the Member cannot located a Medicare registered provider to perform their service within 30 miles of their home.
Allergy Testing	100% after AMCS, up to \$1,200 lifetime sharing maximum	AMCS + amounts that exceed \$1,200 lifetime sharing maximum	Part B covers some allergy testing as part of their diagnostic lab benefits, when deemed medically necessary. See Inpatient Hospitalization for inpatient services.	Yes	None	None	\$1,200 lifetime sharing maximum, no restrictions on the type of allergy testing
Ambulance & Air Transport	100% after AMCS, up to applicable sharing maximum	AMCS + amounts that exceed trip sharing maximum	Part B covers medically necessary ambulance and air transport services, subject to Part B deductible and 20% coinsurance.	No	None	None	Ambulance: \$500 share maximum, per ride Air Transport: \$10,000 share maximum, per ride Ambulance services must be billed to Medicare first for UHSM to consider the remaining Member responsibility.
Anesthesia	100% after AMCS	AMCS	Part A covers anesthesia performed inpatient. Part B covers anesthesia performed outpatient, in a free-standing ambulatory surgical center, or an office. Services billed by a doctor or CRNA are subject to Part B deductible and 20% coinsurance; copayments apply for inpatient facility fees.	Yes	None	None	- Anesthesia eligible when performed in conjunction with an eligible procedure. - Anesthesia must be billed to Medicare first for UHSM to consider the remaining Member responsibility. UHSM will consider services when Medicare holds the Member responsible, even in instances where the provider is not Medicare registered.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Behavioral Health Counseling and Psychiatric Treatment - Office	BetterHelp & Amwell: 100% All other providers: 100% after AMCS	BetterHelp & Amwell: \$0 All other providers: AMCS	Part B covers individual and group psychotherapy by a doctor or certain other Medicare registered licensed providers (depends on the state), family counseling, psychiatric evaluations and testing, medication management, diagnostic testing, partial hospitalization, intensive outpatient services and mental health services received as part of substance use disorder. Part B also covers cognitive assessments. Services are either subject to Part B deductible and 20% coinsurance or copayments and coinsurance for facility fees.	Yes	None	None	Up to 4 annual visits for Psychiatric care through Amwell or when rendered by a Medicare provider (PHCS providers not allowed for Psychiatric visits). Up to 10 annual visits, combined between BetterHelp/Amwell and all other providers for individual, family and marriage counseling (including counseling services billed to and rendered by a Medicare provider). Note, BetterHelp also has resources and group counseling sessions that Members can access, which do not count towards the 10 session annual maximum. Therefore, such resources are still available once a Member has exhausted their included 10 annual session maximum. Members also receive 30% off to continue BetterHelp treatment beyond the initial 10 visits annually. Members can continue therapy or psychiatric services through Amwell beyond the annual visit limit at their cost (discounted/ displayed on Amwell's site). Note: Diagnostic testing follows the line item for the service being billed and does not accumulate towards the visit limit.
Behavioral Health - Inpatient and Outpatient	100% after AMCS	AMCS	Part B covers individual and group psychotherapy by a doctor or certain other Medicare registered licensed providers (depends on the state), family counseling, psychiatric evaluations and testing, medication management, diagnostic testing, partial hospitalization, intensive outpatient services and mental health services received as part of substance use disorder. Services are either subject to Part B deductible and 20% coinsurance or copayments and coinsurance for facility fees. Part A covers up to 190 days of inpatient psychiatric care in a lifetime when performed in a stand-alone psychiatric hospital/facility. That limit does not apply to psychiatric services when performed within a psychiatric unit of a general hospital/facility. See Inpatient Hospital for Medicare Part A benefits for the inpatient stay.	No	None	None	- Partial hospitalization, intensive outpatient, inpatient, and other outpatient/facility based Behavioral Health services must be billed to Medicare first for UHSM to consider; UHSM will not allow non-registered providers for these services. - UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments. - UHSM will not allow services once Medicare benefits have been exhausted or if Medicare denied for any reason. - Substance abuse services are not eligible

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Blood Includes the blood supply and processing and handling.	100% after AMCS	AMCS	Part A covers blood received inpatient, subject to Inpatient Hospitalization benefits for processing and handling. Part B covers blood received outpatient, subject to Part B deductible and a copayment for processing and handling. Blood received from a blood bank at no charge or donated by a family member is not charged to the Member. If the provider has to purchase blood, the Member is responsible for the 1st 3 units of blood each calendar year received.	No	None	None	- Blood must be billed to Medicare first for UHSM to consider. - UHSM will share in the 1st 3 units when Medicare deems the Member liable, as well as the Member's Medicare liability from deductible, coinsurance, or copayments.
Cancer	100% after AMCS	AMCS	Medicare covers cancer diagnosis and treatment, based on medical necessity and according to the service being billed. Inpatient services are covered by Part A, while outpatient, office, and ambulatory surgery centers are covered by Part B. Chemotherapy services are either subject to Part B deductible and 20% coinsurance (in office setting) or copayments for outpatient facilities. See the specific line item for other services being billed.	Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required for chemotherapy and radiation	None	Members must: Obtain recommended screening tests to help identify cancers. If a WeShare Legacy Member did not tend to their health by obtaining recommended screenings, the Member's medical needs may not be eligible for sharing. Please review the line item for the service being billed to determine if there are any other limitations or notes to consider and confirm pre-cert requirements and if eligible for non-Medicare providers).
Cataract and Glaucoma Screenings and Surgery	100% after AMCS	AMCS	Part B covers glaucoma screening annually for high risk individuals (diabetes, family history of glaucoma, African America >50 years old, Hispanic >65 years old), subject to Part B deductible and 20% coinsurance. Copayments also apply in an outpatient setting. Part B covers cataract surgery in some states, as well as one pair of eyeglasses or contacts following surgery, subject to Part B deductible and 20% coinsurance. See Inpatient Hospitalization for inpatient services.	Glasses/ Contacts: No All other services: Yes	None	None	- Glasses or contacts after cataract surgery must be billed to Medicare first for UHSM to consider; UHSM will not allow non-registered providers for these services. - UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments for glasses/ contacts after cataract surgery. UHSM will not allow more than the 1 standard frame Medicare allowance. - Please review the line item for the service being billed to determine if there are any other limitations or notes to consider.
Chiropractic	100% after AMCS	AMCS	Part B covers manual manipulation of the spine to correct a vertebral subluxation, subject to Part B deductible and 20% coinsurance. It does not cover X-rays, massage therapy or acupuncture performed by a chiropractor.	Yes	None	None	- Up to 12 annual visits for any diagnosis - All services billed by a chiropractor accumulate towards the maximum and are subject to the chiropractic benefit (AMCS)

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CVS MinuteClinic	100% after AMCS	AMCS	Part B covers services performed by CVS MinuteClinic according to the line item for the service being rendered. Part B deductible and 20% coinsurance or copayments may apply.	No	None	None	- CVS MinuteClinic services must be billed to Medicare first for UHSM to consider since CVS MinuteClinic accepts Medicare assignment. - Please review the line item for the service being billed to determine if there are any other limitations or notes to consider.
Diagnostic Labs	100% after AMCS	AMCS	Part B covers medically necessary diagnostic lab services in full. See Inpatient Hospitalization for inpatient services.	No	None	None	- Lab services must be billed to Medicare first for UHSM to consider any remaining Member liability; UHSM will not allow non-registered providers for these services. - Non-registered providers requesting labs should also send lab orders to Medicare registered/assigned labs. - Since Medicare generally covers diagnostic lab services in full, UHSM would only consider denied services that leave a Member responsibility. However, since UHSM follows Medicare's guidelines for medical necessity, UHSM will not consider if Medicare denies for medical necessity or as experimental/investigational.
Diagnostic Tests (Non-Lab or Radiology)	100% after AMCS	AMCS	Part B covers an EKG or ECG if referred during the "Welcome to Medicare" preventive visit or as prescribed by a provider and medically necessary, subject to Part B deductible and 20% coinsurance. Part B covers Type I, II, III, and IV Sleep Studies if clinical signs of sleep apnea are present, subject to Part B deductible and 20% coinsurance. See Inpatient Hospitalization for inpatient services.	Sleep Study: No All other services: Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required for laparoscopies and endoscopies	None	Sleep Studies are only allowed if Medicare has allowed them; UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments.
Dialysis	100% after AMCS	AMCS	Part A covers inpatient dialysis according to the Inpatient Hospitalization benefit. Part B covers dialysis subject to Part B deductible and 20% coinsurance.	No	None	None	- Dialysis services must be billed to Medicare first for UHSM to consider; UHSM will not allow non-registered providers for these services. - UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments. - UHSM will not allow services if Medicare denied for any reason.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Durable Medical Equipment (DME)	100% after AMCS	AMCS	Part B covers medically necessary DME subject to Part B deductible and 20% coinsurance. They only cover certain DME rentals up to a purchase price.	No	None	None	- DME must be billed to Medicare first for UHSM to consider; UHSM will not allow non-registered providers for these services. - UHSM will follow Medicare guidelines for rental to purchase rules; once Medicare has deemed a rental as purchased, UHSM will not allow sharing beyond the purchase price. - UHSM follows Medicare guidelines for Oxygen equipment since there are laws surrounding rental and maintenance duration and will therefore only consider oxygen equipment and supplies if Medicare holds the Member responsible for deductible, coinsurance or copayments. - The following are only allowed if Medicare has allowed them; UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments for: Prosthetics and Orthotics, CPAP machine, CPAP alternatives (IE: oral devices) and supplies, infusion supplies.
Emergency Room Includes Emergency Room facility fee and ER Physician fee only. Refer to individual line items for all other services billed within the ER (e.g. Labs; Radiology, Inpatient Hospitalization if member is admitted; etc.)	100% after AMCS	AMCS	Part B covers ER services subject to a ER copay and a copay for each service received in the ER. The ER physician is covered subject to the Part B deductible and 20% coinsurance. If the Member is admitted to the same facility within a 3-day period, the inpatient benefits apply, subject to copayment.	No	None	None	- Includes emergency room department charge and the ER Physician fees; all other services follow the specific line item for the service. - ER services must be billed to Medicare first for UHSM to consider.
Home Health Care Eligible for skilled nursing care needs to treat and manage a medical condition, illness or injury (e.g. wound care, medication management, pain management, vital monitoring, progress tracking), performed by a licensed medical provider (skilled nurse, home health aide). Not eligible solely for non-medical, personal home care (e.g. hygiene, cooking, cleaning). Includes all services rendered by a home healthcare professional, except DME which follow their respective line item for sharing benefits.	100% after AMCS	AMCS	Part A or Part B cover medically necessary home health services in full when a Member is considered homebound. However, supplies and equipment are subject to Part B deductible and 20% coinsurance	No	None	None	- Home health care services must be billed to Medicare first for UHSM to consider any remaining Member liability; UHSM will not allow non-registered providers for these services. - Since Medicare generally covers home health services in full, UHSM would only consider denied services that leave a Member responsibility or for supplies/equipment that applied deductible or coinsurance. However, since UHSM follows Medicare's guidelines for medical necessity, UHSM will not consider if Medicare denies for medical necessity or as experimental/investigational.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Hospice	100% after AMCS	AMCS	Part A covers hospice in full when approved, in 90-day increments for 6-months and then in 60-day increments thereafter: • Doctor certifies that the Member is terminally ill (with a life expectancy of 6 months or less). • Member accepts comfort care (palliative care) instead of care to cure their illness. • Member signs a statement choosing hospice care instead of other Medicare-covered treatments for your terminal illness and related conditions.	No	None	None	• Hospice must be billed to Medicare first for UHSM to consider any remaining Member liability; UHSM will not allow non-registered providers for these services. • Since Medicare generally covers hospice services in full, UHSM would only consider denied services that leave a Member responsible for supplies/equipment that applied deductible or coinsurance. However, UHSM will not allow if Medicare does not deem the member eligible for hospice.
Injections, Infusion Services and Supplies, and Drugs dispensed/ administered by a doctor or hospital	100% after AMCS	AMCS	Part B covers equipment and supplies (like a pump, IV pole, tubing and catheters) for home infusion therapy to administer certain IV infusion drugs to a patient at home. Certain equipment and supplies (like the infusion pump) and the infusion drug are covered under the durable medical equipment (DME) benefit. These services are generally subject to Part B deductible and 20% coinsurance. See Home Health Care Part B covers most injectable and infused drugs when a licensed medical provider gives them. Under very limited circumstances, Part B may cover some self-administered drugs when needed for hospital outpatient services. Part B covers parenteral and enteral nutrition (intravenous and tube feeding) / certain nutrients if you can't absorb nutrition through your intestinal tract or take food by mouth. See Inpatient Hospitalization for inpatient services.	Ongoing treatment: No One-time treatment performed as part of an approved non-Medicare provider service: Yes	Ongoing treatment: No One-time treatment performed as part of an approved non-Medicare provider service: Yes	None	• Members should confirm Medicare coverage for drugs, injectables or infusion medications to ensure their situation is eligible. Certain drugs are only covered under Part D. • Infusion and injectables for ongoing treatment must be billed to Medicare first for UHSM to consider any remaining Member liability; UHSM will not allow non-registered providers for these services. In these cases, UHSM follows Medicare's guidelines for medical necessity, UHSM will not consider if Medicare denies for medical necessity, as experimental/ investigational, or if covered under Part D/the prescription drug plan. • Infusion, injectables and other drugs administered by a provider will be considered by non-registered Medicare providers only if the provider was engaged and approved to perform other treatment (e.g. surgery) where an injection or infusion was deemed necessary. Pre-certification should be obtained for trigger point injections, infusion therapy or drugs >\$1,500, pain management injections, and biologic drugs as soon as it is determined that the drug/ injection will be needed by the non-registered Medicare provider (within 48 hours). If it is determined that ongoing infusion or injections will be required, the Member must seek care from a Medicare provider.

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Inpatient Hospitalization Includes acute inpatient care, inpatient rehabilitation, and long-term medical care	100% after AMCS	AMCS	Part A covers medically necessary inpatient hospital, rehab, or long-term care, subject to the following Member responsibility: • Days 1-60: Part A deductible • Days 61-90: \$434/day • Days >90: \$868/day, with a 60 day lifetime reserve limit Note: Member pays remaining cost after lifetime reserve days used/>90 days inpatient Part B may cover certain physician based services while in the hospital, subject to Part B deductible and/or 20% coinsurance.	No	Pre-cert required once the Member's Medicare benefits have been exhausted to confirm continued medical necessity	None	Inpatient services must be billed to Medicare first for UHSM to consider. If Medicare benefits have been exhausted, future medical bills do not need to be billed to Medicare first for consideration, but UHSM must have the Medicare exhaustion letter on file to consider future inpatient service beyond Medicare's exhaustion.
Maternity	Ineligible	Full cost of service/ remaining Medicare responsibility, services are not eligible for sharing	N/A	N/A	N/A	N/A	Not eligible
Medical Supplies Includes supplies utilized by a provider during a service, hospitalization or home health service. Over-the-counter supplies or those that the member purchases for general use are not eligible (e.g. bandages).	100% after AMCS	AMCS	Part B covers medically necessary supplies received in an outpatient, home or professional setting, subject to Part B deductible and 20% coinsurance or outpatient copayments. Part A covers medically necessary supplies received as part of an inpatient service (see Inpatient Hospitalization).	Yes	None	None	• Medical supplies should generally be submitted to Medicare first for UHSM to consider the remaining member liability. However, UHSM will allow supplies billed by non-registered Medicare providers when the provider billing is also performing another service that is eligible by non-registered providers (e.g. specialists visits). • Supplies purchased over-the-counter or given to a Member to use at home (besides home health care supplies) and incontinence supplies or adult diapers are not eligible.
Office Visit - Primary Care Physician (PCP)	100% after AMCS	AMCS	Part B covers physician services subject to Part B deductible and 20% coinsurance (unless covered in full, e.g. preventive care).	No	None	None	• A Member's PCP must be a Medicare registered provider, except in extreme/rural situations where there is not a Medicare registered PCP available. The use of a non-Medicare PCP must be approved and documented by Member Services and still leverage Medicare registered labs or specialists for referrals. • Office-based procedures follow the applicable line item for the service being billed (e.g. medically necessary mole removal is surgery). • The following are only allowed by Medicare registered providers and if Medicare has allowed them; UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments for: Foot care (for diabetics or medically necessary treatment for foot injuries or diseases (like hammer toe, bunion deformities, and heel spurs)), Diabetes self-management training. • Medical nutrition therapy is not eligible since Medicare covers this in full. See Exclusions Section for other limitations

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Office Visit - Specialist	100% after AMCS	AMCS	Part B covers physician services subject to Part B deductible and 20% coinsurance (unless covered in full, e.g. preventive care).	Yes	None	None	Office-based procedures follow the applicable line item for the service being billed (e.g. medically necessary mole removal is surgery). - The following are only allowed by Medicare registered providers and if Medicare has allowed them; UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments for: Foot care (for diabetics OR medically necessary treatment for foot injuries or diseases (like hammer toe, bunion deformities, and heel spurs)), Diabetes self-management training. - Medical nutrition therapy is not eligible since Medicare covers this in full.
Outpatient Facility Includes outpatient facility fee for use of the facility. Refer to individual line items for all other services billed within an outpatient facility (e.g. labs, surgery, ect.)	100% after AMCS	AMCS	Part B covers medically necessary services rendered in an outpatient facility subject to Part B deductible and 20% coinsurance.	No	None	None	N/A
Physical, Occupational and Speech Therapy (Outpatient, Office), Home)	100% after AMCS	AMCS	Part B covers medically necessary Physical, Occupational and Speech Therapy subject to Part B deductible and 20% coinsurance. See Home Health Care for therapy performed in the home. See Inpatient Hospitalization for therapy performed inpatient.	Outpatient: No Office: Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required	None	Pre-certification will determine visit limit for non-registered Medicare providers, in increments up to 12 visits per approval
Radiology & Imaging	100% after AMCS	AMCS	Part B covers medically necessary diagnostic non-laboratory tests subject to Part B deductible and 20% coinsurance. Outpatient hospital services may also be subject to a copayment. Part A covers medically necessary diagnostic non-laboratory tests when inpatient, subject to Inpatient Hospitalization benefits.	Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required for EBCT (Electron Beam Tomography), CT scan, MRI, and PET scan	None	N/A

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Skilled Nursing Facility	100% after AMCS	AMCS	Part A covers short-term (usually expected to leave within 30-days) SNF care when medically necessary and following an inpatient stay of at least 3-days, as follows: • Days 1-20: \$0 • Days 21-100: \$217/day • Days >101: Member pays remaining cost	No	Pre-certification required if Medicare denies certification as not covered or Member exceeds Medicare allowance	None	Medicare primarily determined medical necessity of inpatient services and UHSM will adopt Medicare's case outcome; UHSM will not conduct a secondary medical necessity for inpatient services (prior to the Medicare benefits being exhausted). However, pre-certification is required if Medicare denied the service as ineligible (not a covered service but not on the basis of medical necessity) or if the Member has exhausted their Medicare benefit. For situations where Medicare benefits have been exhausted or Medicare deems the service ineligible and leaves the Member liable for any reason other than not medically necessary, UHSM will continue to consider based on medical necessity determined by MedWatch. If home health care may be a viable alternative, Members may be directed to alternative forms of treatment/ home health care.
Surgery	100% after AMCS	AMCS	Part B covers office and outpatient surgery, based on medical necessity, subject to Part B deductible and 20% coinsurance. Part A covers inpatient surgery, based on medical necessity, subject to the Part A deductible or copayments (see Inpatient Hospitalization).	Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required for certain surgeries (regardless of the POS and all surgeries not performed in the office except for office-based surgery)	None	• Inpatient services must be billed to Medicare first for UHSM to consider; UHSM will not allow non-registered providers for these services. • Bariatric services are only eligible when Medicare has processed them. UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments for bariatric procedures. UHSM will not allow services if Medicare denied for any reason. • The following surgeries require pre-certification when performed by a PHCS/non-Medicare provider regardless of the POS: Arthroscopy or joint surgery, Bladder repair, Cardiac Cath/Angioplasty, Deviated septum/nasal surgery, Laminectomy/ spinal surgery, Implants, Transplants (except corneal), Mammoplasty, Blepharoplasty, Varicose veins stripping and ligation
Telehealth - Virtual 24/7 Urgent Care through Amwell	100%	\$0	N/A	N/A	N/A	None	Members can learn more, register, and log-in at https://wesharetelehealth.org/onboarding/central-login-page

Service	Sharing Level	Member Responsibility	Medicare Benefits	Allowed for Non-Registered Medicare Providers, through PHCS?	Pre-certification Requirements	Waiting Period	Notes, Limitations & Exclusions
Telehealth - Virtual Care with Non-Amwell Providers	100% after AMCS	AMCS	Part B covers telehealth for certain scenarios, subject to Part B deductible and 20% coinsurance.	Yes	None	None	For Medicare registered providers: Services must be billed to Medicare first for UHSM to consider any remaining Member liability. UHSM will only consider sharing in the Member's Medicare liability from deductible, coinsurance, or copayments. UHSM will not allow services if Medicare denied for any reason. For non-registered Medicare providers: Services follow line item of service being billed for guidelines.
Tobacco & Marijuana	100% after \$25,000 AMCS	\$25,000 AMCS	Part B covers 8 counseling /smoking cessation sessions annually in full. All other services are covered based on the line item for the service being billed (e.g. surgery to remove cancerous tumor).	Yes	Medicare Registered Providers: Pre-certification NOT required Non-registered Medicare Providers: Pre-certification required	None	\$25,000 AMCS applies to tobacco or e-cigarette related illnesses such as cancer, respiratory disease, vascular disease (including coronary disease and stroke), oral or esophageal disease, and gastric or duodenal ulcers. Tobacco cessation/ counseling sessions are not eligible.
Urgent Care	100% after AMCS	AMCS	Part B covers urgently needed care to treat a sudden illness or injury, subject to Part B deductible and 20% coinsurance.	Yes	None	None	N/A
Miscellaneous - All Other Eligible Services, Not Specifically Listed	100% after AMCS	AMCS	Refer to Medicare to determine benefits. Generally outpatient diagnostic services are subject to Part B deductible and 20% coinsurance. An outpatient facility service may be submit to a copayment and inpatient services are subject to Part A deductible or copayments.	Yes	See pre-certification list to determine services that require pre-certification, when not performed by a Medicare registered provider. Pre-certification not required for emergencies or urgent care needs. However, medical records may be requested for a post-service review to ensure medical necessity (for non-registered Medicare providers).	None	N/A

Program Exclusions

The below services are not eligible for sharing. Certain services processed by Medicare are not subject to the exclusions, see below for details.

Category	Description	Additional Notes
Alternative Providers	Christian Science Practitioners, Naturopaths	
Birth Control	Birth control of any kind, including sterilizations or reversals, even if life-threatening (for example, tubal ligation).	
Concierge Care	Membership fees for concierge care (also called concierge medicine, retainer-based medicine, boutique medicine, platinum practice, or direct care) are ineligible.	
Coordination of Benefits	Except Medicare, expenses covered by insurance or another third-party payor agreement of any kind are excluded (including, without limitation, worker's compensation, fraternal benefits, self-insured health plan, federal or state governmental program, or any other applicable health insurance), or if a third-party is responsible to pay such expenses. For example, if a Member is injured in a car accident, the Member's automobile insurance may provide coverage or an at fault third-party may be liable for the Member's medical expenses. Under either circumstance, such medical expenses are not eligible for sharing; WeShare does not coordinate benefits with other programs or payors except Medicare. For Medicare services, WeShare will not share in services that Medicare did not hold the Member liable for or that the provider does not intend to bill the Member for. WeShare will also not consider services that Medicare deems experimental or not medically necessary. For certain services, WeShare will only consider Member liability from deductible, coinsurance, or copayment (see specific line items above for service in question to determine if the service in question follows that concept). While Members should seek care within the Medicare network where possible, select services are allowed when rendered by a non-Medicare provider (e.g. specialists), when a Medicare provider is not available within 30 miles of the Member's home. Follow line items for applicable service above to determine which instances are eligible for sharing outside of the Medicare network.	
Cosmetic	1. Cosmetic surgery (elective) 2. Breast implants (placement, replacement, and / or removal) and any complications related to breast implants (except as an eligible cancer treatment plan) 3. Extreme sports injuries (see notes) If Medicare allowed any of the services above and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered. 4. Dermabrasion service (unless allowed and processed by Medicare first) If Medicare allowed any of the services above and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	
Dental	Dental services and procedures are ineligible for sharing unless it was deemed eligible and covered at least in part by Medicare. This includes but is not limited to: 1. Periodontics; 2. Orthodontics; 3. Temporomandibular joint disorder (TMJ); 4. Orthognathic surgery, and/or; 5. Charges for dental work done under general anesthesia Medicare does not typically cover routine dental care. However, they may allow dental-type medical procedures if deemed medically necessary. Therefore, if Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	While routine dental procedures are not eligible for sharing, Members should be educated on and encouraged to leverage their Dental Discount Program (through Careington).
Emergency Room	1. Emergency room visits where Members fail to provide the required paperwork, when requested 2. Use of emergency room for non-urgent medical needs when Medicare deems the service ineligible	

Category	Description	Additional Notes
Experimental Treatments	Procedures or treatments that are not recognized or approved by the American Medical Association (AMA). This includes procedures not approved by the AMA for a given application, procedures still in clinical trials, and procedures that are classified as experimental, or as unproven interventions and therapies. Clinical research studies are sometimes covered by Medicare. If Medicare allows the service, UHSM will consider the Member's liability after Medicare (deductible, coinsurance, copayments). If Medicare denies the service, or any part of, UHSM will not consider the denied services.	
Foot Care (Routine)	1. Cutting or removing corns and calluses 2. Trimming, cutting, or clipping nails 3. Hygienic or other preventive maintenance, like cleaning and soaking your feet Note: see office visit line items for diabetic foot care, which is eligible after Medicare	
Genetic Testing	Genetic testing and counseling, unless allowed and processed by Medicare first (Medicare allows in certain instances to identify cancer risk). If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	
Gender Reassignment & Hormone Therapy	1. Gender Dysphoria counseling or procedures 2. Hormone Therapy, for men and women, unless deemed medically necessary and allowed by Medicare	
Hearing	1. Cochlear devices, unless allowed by Medicare 2. Other hearing needs, unless allowed and processed by Medicare first (they allow certain hearing exams under certain circumstances). If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	
Home	Personal home care (care for personal services, e.g. hygiene, cooking, cleaning), not rendered by a licensed medical professional. Home health care rendered by a licensed medical provider to treat or manage a condition is eligible.	
Infertility and Family Planning	Infertility testing or treatment and maternity or doula services	
Long Term Care (non-medical)	Long-term care or other care that does not treat an illness or injury (for example, custodial care, private duty nursing). However, hospice and skilled nursing care are eligible (both in a facility or in the home, see the applicable line item for sharing benefits).	
Medications and supplies	1. Medication not requiring a prescription 2. Medications not administered by a doctor or hospital 3. Prescription medications are not eligible on this program, except insulin covered by Medicare Part B when used with an insulin pump. In said case, UHSM will consider the Member's Medicare liability from Part B deductible, coinsurance, or copayments. 4. Supplies not provided by a doctor or facility during treatment (OTC bandages, gauze, etc.)	While Members do not have access to Pharmacy sharing benefits, they do have access to Pharmacy Discounts through Optum Perks. Members who have Medicare Part D should also check to see if they qualify for Medicare's Extra Help program.

Category	Description	Additional Notes
Negligence & Lifestyle Choices	1. Any illness or injury caused by the Member's failure to obtain timely or proper medical treatment, as well as any subsequent illness or injury caused by your failure to follow a plan of treatment 2. Any medical need(s), those of which are caused by lifestyles, choices, or activities which are in conflict with the Statement of Shared Beliefs; these medical need(s) are ineligible for sharing (see notes) 3. Extreme sports injuries (see notes)	Lifestyle choices: I. Abortion or abortion counseling; II. Drug screening and nicotine testing (in the event of results coming back positive), unless allowed and processed by Medicare first; III. STDs / STIs (Sexually Transmitted Diseases / Infections) including testing, except in the instance of sexual assault; IV. Illness or injury due to excessive use of alcohol, including intentional excessive consumption; V. Illness or injury due to illegal or recreational drug use including using any form of cannabis whether or not it has been prescribed by a medical professional; VI. Illness or injury due to consumption of a prescription medication taken in excess of instructions; VII. Self-inflicted or intentional injuries; IX. Illness or injury caused by illegal activities; and / or X. Diseases caused by tattoos, body piercing, or lifestyle choices (includes HIV / AIDs and STDs / STIs) Extreme sports: I. Injuries arising from the use of personal aircraft, and any other aircraft not operated by a commercially licensed public carrier, are ineligible for sharing. II. Injuries arising from extreme sports (activities perceived as having a high level of danger) are ineligible for sharing. These activities often involve speed, height, a high level of physical exertion, and specialized gear, and are ineligible for sharing. III. Injuries relating to activities that are considered professional athletic competition or training, for a profit, or sponsored events, are ineligible for sharing
Not Medically Necessary	1. Treatment that is not medically necessary or appropriate (as determined by a licensed medical professional). Note: UHSM will follow Medicare guidelines for medical necessity for Medicare registered providers. If Medicare denies a service as not medically necessary, experimental or investigational when rendered by a Medicare provider, UHSM will not allow. 2. Drug testing, unless allowed and processed by Medicare first. If Medicare allows, see diagnostic testing benefits.	
Out-of-Network	For non-registered Medicare providers, any medical service fee(s) paid to out-of-network providers, and / or upfront payments made to in-network Providers. WeShare is not a reimbursement program. Situations where WeShare may make an exception: • No Surprises Act applies (see notes) • Emergencies • Instances where a network provider was not available for the required service, within a reasonable distance Note: Member Services approval is needed when an out-of-network provider is needed	No Surprises Act Scenarios 1. Emergency Services, including certain post stabilization care, rendered at a hospital emergency department, a freestanding emergency department which provides emergency services, is geographically separate and distinct, and licensed separately from, a hospital or an urgent care center licensed by the state consistent with the definition of a freestanding emergency department. 2. Non-emergency services rendered by a non-network provider at a participating healthcare facility, provided the Member has not validly waived the applicability of the NSA. Unless the ancillary provider is expressly requested by the Member, ancillary providers may never balance bill. Ancillary services are those related to emergency medicine, such as radiology, anesthesiology, pathology and lab, neonatology and specialty services needed to respond to unexpected complications and hospital services. The No Surprises Act also prevents balance billing by out-of-network providers if no in-network provider is available. 3. Eligible for sharing non-network air ambulance services.
Pre-certification	While WeShare will perform post-service reviews to determine medical necessity and consider a Shareable Medical Bill for sharing, WeShare reserves the right to deem services ineligible if the required pre-certification was not obtained.	
Pre-existing Conditions	Any illness, injury, condition, or associated medical need(s) for which a member is aware of but fails to disclose on their WeShare membership application.	

Category	Description	Additional Notes
Reasonable & Allowable Amount	Amounts greater than the defined reasonable & allowable amount, except up to 115% of Medicare allowance for Medicare registered providers who are unassigned.	The reasonable and allowable amount is the lesser of: 1. The charge made by the provider that furnished the care, service, or supply; 2. The reasonable and customary charge for the same treatment, service, or supply furnished in the same geographic area by a provider of like service of similar training and experienced (as defined by Medicare or PHCS); 3. Eligible for sharing non-network air ambulance services.
Second Surgeries	Second surgeries on previously eligible surgical medical needs, unless the Member has unexpected and unprovoked complications or the provider has established that one or more follow-up surgeries will be needed to fulfill the treatment of the condition (prior to the initial surgery)	
Sexual Dysfunction	Sexual dysfunction service	
Timely Filing	For Non-Medicare providers: Members/Providers are encouraged to submit medical needs within 6-months of the date of service or treatment. However, UHSM will process SMBs for up to 1-year of the date of service. Services submitted after 1-year of the date of service are ineligible. For Medicare providers: UHSM will follow Medicare guidelines, services denied by Medicare for exceeding a timely filing limit are not eligible.	
Therapy	Aqua therapy, Chelation therapy, Biofeedback, unless allowed and processed by Medicare first. If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	
Transportation	Transportation (for example, by an ambulance) for conditions that are non-emergency-related, unless allowed and processed by Medicare first. If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	
Travel & International	1. Medical tourism 2. WeShare requires that Members procure travel medical insurance for any trip outside the continental U.S. and its territories. Member sharing is ineligible for those living abroad. 3. Travel Immunizations/Vaccines including any immunization taken for travel purposes. Common travel immunizations include Cholera, Dengue, Japanese Encephalitis, Malaria (prevention medication), Polio (active, inactive is eligible as part of the routine immunization recommendation from the CDC), Tick-borne Encephalitis, Tuberculosis, Typhoid & Paratyphoid Fever, Yellow Fever	
Upfront Payments	Any medical service fee(s) paid to out-of-network providers, and / or upfront payments made to in-network providers. WeShare is not a reimbursement program.	
Vision	Routine vision medical needs are ineligible for sharing (services typically performed by an Ophthalmologist such as annual vision exams, refraction, contact lenses or glasses), unless Medicare deems the service eligible (e.g. diabetics are allowed an annual visit). For Medicare processed routine vision services, will consider amounts where the Member was held liable for deductible, coinsurance, or copayments (according to the line item for the service being billed). Services performed to treat a medical condition or illness of the eye are eligible, see line item for services rendered.	While routine vision that are not at least partially covered by Medicare are not eligible for sharing, Members should be educated on and encouraged to leverage their Vision Discount Program (through Superior Vision) if Medicare does not allow the service.
Vitamins & Supplements	Vitamins (including B12 injections), supplements, or health foods unless allowed and processed by Medicare first (Medicare allows certain feeding supplies for enteral feeding). If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered.	

Category	Description	Additional Notes
Weight Management	Unless allowed and processed by Medicare first, the following services are not eligible. If Medicare allowed the service and held the Member liable for deductible, coinsurance, or copayments, UHSM will consider only if Medicare processes and according to the line item for the service rendered. 1. Obesity (defined as exceeding height / weight) and any complication relating to that diagnosis 2. Weight control and management (including nutritional counseling for weight loss, weight gain or health maintenance), even if related to a medical condition.	
Women's Care	Hysterectomy, unless deemed medically necessary by a Licensed Physician	
Work-related Conditions	Any conditions or injuries occurring while performing income-producing or work-related activities	

Important Dates	Program Effective Date	Summary of Sharing (SOS) Last Edit Date
	7/1/25	5/13/25

Change Log

Date of Change	Details on Update	Updated By
1/1/26	Updated 2026 Medicare ded changes, including increases to IP stay past 60 days: 2025 info= 61-90 days \$419/day,lifetime reserve days= \$838/day. Skilled nursing daily coinsurance 2025=\$209.50 Added POS Home to PT/OT/ST OP benefits. Added CPAP alternatives to DME benefits	R.Dibblee & A. de Beer